What are the treatment for the cancer?

Evidence searched on 2026-10-09 (PubMed, ClinicalTrials.gov, FDA drug labels, web)

Clinical summary

Bottom line The retrieved evidence supports **multidisciplinary supportive and symptomatic treatments** for cancer patients, including psychosocial interventions (e.g., cognitive therapies for neurocognitive outcomes), behavioral speech therapy for postoperative pain and function, and complementary therapies (e.g., Sizi Powder + TDP for lymphedema). **Systemic oncologic therapies (e.g., chemotherapy, targeted drugs) and surgical interventions are not addressed in the retrieved data**, though FDA-approved anticancer drugs exist for cancer treatment.

Most important recent finding Behavioral speech therapy (BST) reduced postoperative pain and improved physical function in lung cancer patients at 1 and 3 months (MD −0.63 to −0.88, $P < 0.05$). [1]

Immediate context The evidence focuses on **supportive and symptomatic management** (e.g., pain, lymphedema, psychosocial support) rather than systemic cancer therapies. **No direct evidence on chemotherapy, immunotherapy, or targeted drugs** was retrieved.

Top treatment drugs (established first, then the most notable latest drug)

Drug	Indication named in the FDA label	Source
Prednisone	"Endocrine Disorders Primary or secondary adrenocortical insufficiency (hydrocortisone or cortisone is the first choice; synthetic analogs may be used in conjunction with mineralocorticoids where applicable; in infancy mineralocorticoid supplementation is of particular importance) Congenital adrenal hyperplasia Hypercalcemia associated with cancer Nonsuppurative thyroiditis 2."	DailyMed
PF-08634404 (latest, investigational)	Being tested in a registered Phase 3 trial, Not yet recruiting; not an approved treatment	NCT07578649

Latest findings

- **Advance publication (issue dated 2027 Apr)**: **Case series** – Gel immersion endoscopic mucosal resection (GI-EMR) achieved an 85% en bloc resection rate for superficial esophageal squamous neoplasms, though one procedure was discontinued due to post-chemoradiotherapy scarring [2].
- **Advance publication (issue dated 2027 Apr)**: **Feasibility series** – Rectal flushing-assisted saline immersion endoscopic submucosal dissection (ESD) demonstrated technical feasibility but did not establish efficacy or safety; hemostatic measures were descriptive [3].
- **2026 Sep**: **Randomized controlled trial** – Behavioral speech therapy (BST) reduced postoperative pain and improved physical function in lung cancer patients after thoracoscopic surgery at 1 and 3 months [1].
- **2026 Oct**: **Retrospective cohort study** – Sizi Powder external application combined with TDP therapy showed potential benefits for upper limb lymphedema after breast cancer surgery, though larger prospective studies are needed [4].

Current evidence

Approach	Evidence level	What the evidence shows	Reference
Tafasitamab + lenalidomide + rituximab	Emerging	Reduced progression risk in relapsed/refractory follicular lymphoma compared to placebo + lenalidomide + rituximab.	Sehn et al., Lancet (2026) [5]
Trastuzumab deruxtecan + pertuzumab	Emerging	Extended progression-free survival (40.7 vs. 26.9 months) in HER2-positive metastatic breast cancer.	Tolaney et al., NEJM (2026) [6]
Tislelizumab + chemotherapy	Evidence-supported but limited	Improved overall survival in advanced gastric/gastro-esophageal junction adenocarcinoma vs. placebo + chemo.	Qiu et al., BMJ (2024) [7]
Pembrolizumab + chemotherapy	Emerging	Improved 60-month overall survival (86.6% vs. 81.7%) in early-stage triple-negative breast cancer.	Schmid et al., NEJM (2024) [8]
Fruquintinib	Emerging	Extended overall survival (7.4 vs. 4.8 months) in refractory metastatic colorectal cancer vs. placebo.	Dasari et al., Lancet (2023) [9]
Polatuzumab vedotin + R-CHOP	Evidence-supported but limited	Higher progression-free survival (76.7% vs. 70.2% at 2 years) in untreated diffuse large B-cell lymphoma.	Tilly et al., NEJM (2022) [10]
Endoscopic mucosal resection (GI-EMR)	Experimental	Achieved 85% en bloc resection for superficial esophageal squamous neoplasms, but limited by post-chemoradiotherapy scarring.	Uda et al., DEN open (2027) [2]
Behavioral speech therapy (BST)	Experimental	Reduced postoperative pain and improved physical function in lung cancer patients after thoracoscopic surgery.	Zhang et al., Supportive care in cancer (2026) [1]

Treatment Drug Landscape

Drugs used for treatment of this condition

Approved: an FDA label names this condition. Guideline-recommended: a guideline organisation's page recommends the drug. Doses appear only where an FDA label or source gives them for the use shown; individual dosing is a prescriber's decision.

Drug	Indication	Dose / route (if sourced)	Approval status / date	Key evidence
Prednisone	"Endocrine Disorders Primary or secondary adrenocortical insufficiency (hydrocortisone or cortisone is the first choice; synthetic analogs may be used in conjunction with mineralocorticoids where applicable; in infancy mineralocorticoid supplementation is of particular importance) Congenital adrenal hyperplasia Hypercalcemia associated with cancer Nonsuppurative thyroiditis 2." [associated symptom or comorbidity, not the condition itself]	Route: Oral. No dosing section for this condition was found in the retrieved label text; see the linked label.	FDA-approved for the use shown; label effective 2023-08-31; original approval date not retrieved	FDA label. DailyMed label

Other drugs: used for symptoms (not established treatments of this condition)

Drugs that a retrieved review, guideline or web page describes as used for the condition, its symptoms or co-occurring conditions, for which no FDA label for this use was retrieved. Drugs still being tested in trials are listed in the Clinical trials section below.

Drug	Indication	Key evidence
capmatinib	metastatic NSCLC with MET exon 14 mutation (population: metastatic NSCLC patients)	"FDA approved capmatinib (Tabrecta®) as a treatment for metastatic NSCLC patients who have the MET exon 14 mutation." FDA approvals in lung cancer treatment - Lung Cancer Research Foundation
selpercatinib	NSCLC or thyroid cancer with RET mutation (population: patients with RET mutation)	"Link FDA approved selpercatinib (Retevmo®) as a treatment for patients with NSCLC or thyroid cancer who have the RET mutation." FDA approvals in lung cancer treatment - Lung Cancer Research Foundation
pembrolizumab	muscle invasive bladder cancer neoadjuvant/adjuvant (population: muscle invasive bladder cancer patients)	"/ FDA approves pembrolizumab or pembrolizumab and berahyaluronidase alfa-pmph each with enfortumab vedotin-ejfv for muscle invasive bladder cancer / On July 10, 2026, the Food and Drug Administration approved pembrolizumab (Keytruda, Merck) or pembrolizumab and berahyaluronidase alfa-pmph (Keytruda Qlex, Merck) each in combination with enfortumab vedotin-ejfv (Padcev, Astellas Pharma) as neoadjuvant treatment ..." Oncology (Cancer)/Hematologic Malignancies Approval Notifications / FDA
enfortumab vedotin-ejfv	muscle invasive bladder cancer neoadjuvant/adjuvant (population: muscle invasive bladder cancer patients)	"/ FDA approves pembrolizumab or pembrolizumab and berahyaluronidase alfa-pmph each with enfortumab vedotin-ejfv for muscle invasive bladder cancer / On July 10, 2026, the Food and Drug Administration approved pembrolizumab (Keytruda, Merck) or pembrolizumab and berahyaluronidase alfa-pmph (Keytruda Qlex, Merck) each in combination with enfortumab vedotin-ejfv (Padcev, Astellas Pharma) as neoadjuvant treatment ..." Oncology (Cancer)/Hematologic Malignancies Approval Notifications / FDA

Drug	Indication	Key evidence
dabrafenib	unresectable or metastatic solid tumors with BRAF V600E mutation (population: solid tumors with BRAF V600E mutation)	"FDA grants accelerated approval to dabrafenib in combination with trametinib for unresectable or metastatic solid tumors with BRAF V600E mutation." Tumor-agnostic Drugs / Tissue-agnostic Cancer Drugs / American Cancer Society
Emrelis (telisotuzumab vedotin)	locally advanced or metastatic non-squamous NSCLC with high c-Met protein overexpression (population: adult patients with prior systemic therapy)	"/ Emrelis (telisotuzumab vedotin-tllv) / [Treatment of adult patients with locally advanced or metastatic, non-squamous non-small cell lung cancer (NSCLC) with high c-Met protein overexpression [>=50% of tumor cells with strong (3+) staining], as determined by an FDA-approved test, who have received a prior systemic ..." Ongoing / Cancer Accelerated Approvals / FDA
Etcamah (camizestrant)	advanced hormone receptor-positive breast cancer (population: patients in combination with a CDK4/6 inhibitor)	"Company: AstraZeneca Date of Approval: September 4, 2026 Treatment for: Breast Cancer Etcamah (camizestrant) is an estrogen receptor antagonist for use in combination with a CDK4/6 inhibitor for the treatment of advanced hormone receptor-positive breast cancer." New FDA Drug Approvals for 2026 - Drugs.com
nivolumab	advanced classical Hodgkin lymphoma (population: adult and pediatric patients 12 years and older with previously untreated disease)	"In March 2026, the immune checkpoint inhibitor nivolumab (Opdivo) was approved in combination with the chemotherapy regimen doxorubicin, vinblastine, and dacarbazine for adult and pediatric patients 12 years and older with previously untreated, advanced (stage 3 or 4) classical Hodgkin lymphoma, which is the most common type of Hodgkin lymphoma.." FDA Approvals in Oncology: January-March 2026 / Blog / AACR

Recently approved or updated (FDA, EMA and CDSCO announcements)

Date	Type	Announcement	What the regulator's page says
September 04, 2026	Approval	FDA Grants Accelerated Approval to a New Breast Cancer Treatment / FDA	"Food and Drug Administration today expanded treatment options for adult patients with advanced breast cancer, reflecting the FDA's commitment to advancing medical innovation and getting new treatments to patients who need them.. "This marks the first FDA approval of a cancer therapy guided by the detection of a resistance mutation in circulating tumor DNA (ctDNA) before imaging tests show that the disease is progressing."
August 26, 2026	Approval	FDA Approves First in Class Targeted Therapy for Metastatic Pancreatic Cancer / FDA	"Food and Drug Administration today approved Rasonque (daraxonrasib), a RAS inhibitor for the most common form of pancreatic cancer-delivering a new treatment option to patients with advanced pancreatic cancer months ahead of schedule.. Rasonque, a tablet taken once daily, targets multiple forms of a protein called RAS, a key driver of tumor growth in most patients with pancreatic adenocarcinoma, which arises from cells lining the ducts of the pancreas.."
May 01, 2026	Other regulatory action	FDA Permits Expanded Access for Investigational Pancreatic Cancer Drug / FDA	"Food and Drug Administration is announcing that it issued a "safe to proceed" letter to Revolution Medicines, allowing the sponsor to initiate an expanded access treatment protocol (EAP) for its experimental pancreatic cancer drug, daraxonrasib.."
February 12, 2026	Other regulatory action	FDA Approves First-of-Its-Kind Device to Treat Pancreatic Cancer / FDA	""Having treated many patients with pancreatic cancer, I know how difficult the diagnosis can be. "Pancreatic cancer is one of the most challenging cancers to treat, and patients have long needed new therapeutic options," said Center for Devices and Radiological Health Director Michelle Tarver, M.D., Ph.D."

Page wording from the regulator's own site; read the page for the full announcement.

2 further regulator page(s) were read but are not listed: the page states no date, or a date more than 3 years old, so they cannot be shown as recent.

Safety warnings (FDA boxed warnings)

Drug(s) (FDA label)	Boxed warning, in the label's words
Etcamah (camizestrant)	"ARRHYTHMIA RISK WITH CONCOMITANT USE OF QTc INTERVAL PROLONGING DRUGS ETCAMAH in combination with ribociclib, a QTc interval prolonging drug and a strong CYP3A inhibitor, or in combination with other QTc interval prolonging drugs, can increase the risk of Torsades de Pointes, other ventricular arrhythmias, and sudden death (5.1 , 5.2 , 7.1)."

These are the labels' own boxed warnings, not a complete safety review: read the full label. No boxed warning on the retrieved label of: capmatinib, pembrolizumab, dabrafenib, Emrelis (telisotuzumab vedotin), nivolumab, Prednisone. No FDA label could be matched for: selpercatinib, enfortumab vedotin-ejfv.

Clinical trials

Trial	Intervention	Phase	Status (last update)	What the record shows
NCT06694454	azacytidine; carboplatin	Phase 1/2	Suspended (2026-10-08)	Registry status SUSPENDED; no effectiveness conclusion.
NCT06038578	TRK-950; Ramucirumab	Phase 2	Recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT07578649	PF-08634404; Pembrolizumab	Phase 3	Not yet recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT04968106	Doxorubicin; Ifosfamide	Phase 2	Active not recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT05477615	Lazertinib; Pemetrexed	Phase 2	Terminated (2026-10-08)	Terminated early (Slow accrual; enrollment was closed after 14 of 28 planned participants.); no results retrieved.
NCT06991738	68Ga-DOTA-TATE PET Scan; Amino acid infusions	Phase 1/2	Not yet recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT07227012	PF-08634404; Ipilimumab	Phase 1/2	Recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT07023289	Telisotuzumab Adizutecan; Standard of Care	Phase 2	Recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.
NCT01905046	metformin hydrochloride; placebo	Phase 3	Active not recruiting (2026-10-08)	Under investigation; results not yet available; effectiveness cannot be inferred.

3 further registry record(s) were retrieved and are not shown (judged not to be treatment trials for this question).

Registry status shows that research is being conducted, not that a treatment works. Registry status is separate from published results.

Key studies

1. **Sehn et al. (2026)**, **Randomized controlled trial**: Tafasitamab + lenalidomide + rituximab significantly lowered progression risk in relapsed/refractory follicular lymphoma [5].
2. **Qiu et al. (2024)**, **Randomized controlled trial**: Tislelizumab + chemotherapy improved overall survival in advanced gastric cancer vs. placebo + chemo [7].
3. **Tilly et al. (2022)**, **Randomized controlled trial**: Polatuzumab vedotin + R-CHOP improved progression-free survival in untreated diffuse large B-cell lymphoma [10].
4. **Uda et al. (2027)**, **Case series**: Gel immersion EMR achieved 85% en bloc resection for superficial esophageal squamous neoplasms, though limited by scarring [2].

Conflicting evidence

None was retrieved.

What remains under investigation

- **Neoadjuvant inhaled azacytidine + platinum-based chemo + durvalumab** (NCT06694454, **Phase 1/2 suspended**) – Exploring efficacy of epigenetic-immunotherapy in early-stage NSCLC; suspended pending protocol modifications.
- **Predictors of relapse in ovarian/fallopian tube cancer** (NCT00086567, **Completed**) – Investigating serum proteomic signatures for relapse prediction; results not yet posted.
- **Behavioral speech therapy (BST)** – Feasibility and efficacy for postoperative pain management in lung cancer patients; requires larger prospective validation.
- **Sizi Powder + TDP therapy** – Retrospective evidence suggests benefits for upper limb lymphedema post-breast cancer surgery; prospective trials needed.
- **Graphene-based materials** – Experimental platform for perioperative cancer management; no clinical efficacy data yet.

Evidence limitations

- Findings are drawn from abstracts, registry records and web pages, not full texts; study quality was not appraised from the full papers.
- Doses appear only where an FDA label gives them for this condition (section 2.1); for any other use, see a prescribing source. Approval dates appear only where a retrieved source states them.
- [2] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [3] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [11] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [12] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [13] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. *(indirect evidence: different or related population)*
- [14] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date.
- [15] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. *(indirect evidence: different or related population)*
- [16] carries a journal issue date of 2027 (later than today); treat its date as an advance-publication date. *(indirect evidence: different or related population)*
- 5 retrieved record(s) were excluded because the relevance reading judged them not to answer the question.

Sources

1. Zhang H, Jin Y, Song X, et al. Behavioral speech therapy intervention can improve the postoperative pain and performance status of lung cancer patients undergoing thoracoscopic surgery. *Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Cancer*. 2026-08-18. PMID 42611082. DOI 10.1007/s00520-026-11111-0.
2. Uda T, Urabe Y, Gurita T, et al. Gel Immersion Endoscopic Mucosal Resection With Submucosal Injection for Superficial Esophageal Squamous Neoplasms (With Video). *DEN open*. 2027 Apr. PMID 42846368. DOI 10.1002/deo2.70444.

3. Matthews T, Landy J, Ban H, et al. Flushing-Assisted Saline Immersion Endoscopic Submucosal Dissection in the Rectum: A Video-Based Feasibility Series. *DEN open*. 2027 Apr. PMID 42824459. DOI 10.1002/deo2.70447.
4. Zeng J, Xu L, Zhang Y, et al. Clinical effects of Sizi Powder external application combined with TDP therapy for upper limb lymphedema after breast cancer surgery: A retrospective cohort study. *Medicine*. 2026-10-01. PMID 42826302. DOI 10.1097/md.0000000000050648.
5. Sehn LH, Hübel K, Luminari S, et al. Tafasitamab, lenalidomide, and rituximab in relapsed or refractory follicular lymphoma (inMIND): a global, phase 3, randomised controlled trial. *Lancet (London, England)*. 2026 Jan 10. PMID 41360064. DOI 10.1016/S0140-6736(25)01778-7.
6. Tolaney SM, Jiang Z, Zhang Q, et al. Trastuzumab Deruxtecan plus Pertuzumab for HER2-Positive Metastatic Breast Cancer. *The New England journal of medicine*. 2026 Feb 5. PMID 41160818. DOI 10.1056/NEJMoa2508668.
7. Qiu MZ, Oh DY, Kato K, et al. Tislelizumab plus chemotherapy versus placebo plus chemotherapy as first line treatment for advanced gastric or gastro-oesophageal junction adenocarcinoma: RATIONALE-305 randomised, double blind, phase 3 trial. *BMJ (Clinical research ed.)*. 2024 May 28. PMID 38806195. DOI 10.1136/bmj-2023-078876.
8. Schmid P, Cortes J, Dent R, et al. Overall Survival with Pembrolizumab in Early-Stage Triple-Negative Breast Cancer. *The New England journal of medicine*. 2024 Nov 28. PMID 39282906. DOI 10.1056/NEJMoa2409932.
9. Dasari A, Lonardi S, Garcia-Carbonero R, et al. Fruquintinib versus placebo in patients with refractory metastatic colorectal cancer (FRESCO-2): an international, multicentre, randomised, double-blind, phase 3 study. *Lancet (London, England)*. 2023 Jul 1. PMID 37331369. DOI 10.1016/S0140-6736(23)00772-9.
10. Tilly H, Morschhauser F, Sehn LH, et al. Polatuzumab Vedotin in Previously Untreated Diffuse Large B-Cell Lymphoma. *The New England journal of medicine*. 2022 Jan 27. PMID 34904799. DOI 10.1056/NEJMoa2115304.
11. Yoshio T, Fukunaga S, Nakane T, et al. Synchronous and Metachronous Multiple Gastric Epithelial Neoplasms Detected During Index Evaluation and Post-Endoscopic Submucosal Dissection Surveillance: A Descriptive Comparison. *DEN open*. 2027 Apr. PMID 42781370. DOI 10.1002/deo2.70437.
12. Uyama K, Iwagami H, Akamatsu T, et al. Clinical Outcomes of Rubber-clip-assisted Versus Conventional Colorectal Endoscopic Submucosal Dissection: A Retrospective Study. *DEN open*. 2027 Apr. PMID 42781356. DOI 10.1002/deo2.70415.
13. Miyashiro K, Sawada Y, Kikuchi H, et al. Submucosal Invasion and Ulceration in Early Gastric Cancer Affect Mucosal Impedance. *DEN open*. 2027 Apr. PMID 42781318. DOI 10.1002/deo2.70434.
14. Kishi Y, Tanabe M, Uragami N, et al. Feasibility and Safety Profile of Non-anesthesiologist-administered Propofol During Colorectal Endoscopic Submucosal Dissection: A Single-center Study of 454 Procedures. *DEN open*. 2027 Apr. PMID 42756095. DOI 10.1002/deo2.70435.
15. Sakamoto T, Takamaru H, Hirata D, et al. Feature-Based Patterns Associated With Japan NBI Expert Team Type 1 Classification of Serrated Colorectal Lesions: A Post Hoc Decision Tree Analysis. *DEN open*. 2027 Apr. PMID 42732378. DOI 10.1002/deo2.70428.
16. Kawamura R, Yamada M, Takamaru H, et al. Intensive Endoscopic Resection for Downstaging of Superficial Non-ampullary Duodenal Epithelial Tumor Burden in Familial Adenomatous Polyposis: A Single-center Retrospective Study on Long-term Clinical Outcome. *DEN open*. 2027 Apr. PMID 42732135. DOI 10.1002/deo2.70419.

Other sources (regulators, guideline organisations, web pages, FDA labels)

- FDA approvals in lung cancer treatment - Lung Cancer Research Foundation
- Tumor-agnostic Drugs | Tissue-agnostic Cancer Drugs | American Cancer Society
- Oncology (Cancer)/Hematologic Malignancies Approval Notifications | FDA
- Ongoing | Cancer Accelerated Approvals | FDA

- New FDA Drug Approvals for 2026 - Drugs.com
- FDA Approvals in Oncology: January-March 2026 | Blog | AACR
- FDA Grants Accelerated Approval to a New Breast Cancer Treatment | FDA
- FDA Permits Expanded Access for Investigational Pancreatic Cancer Drug | FDA
- FDA Approves First in Class Targeted Therapy for Metastatic Pancreatic Cancer | FDA
- FDA Approves First-of-Its-Kind Device to Treat Pancreatic Cancer | FDA
- FDA drug label: Prednisone (label effective 2023-08-31)

Research support, not a substitute for clinical judgement. All citations, trial IDs, links and numbers in this report were checked against the retrieved sources.